

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

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Finding reasoning paths:

- 1. Inattention, hyperactivity, impulsivity, forgetfulness -> Attention Deficit Hyperactivity Disorder (ADHD)
- 2. Dysfunctional family environment -> Behavioral issue
- 3. Impairments in social interactions -> Social interaction and social reasoning -> ADHD

Reasoning Process:

- 1. **Symptoms Analysis:** The 5-year-old boy presents with inattention, hyperactivity, impulsivity, and forgetfulness. These symptoms are characteristic of Attention Deficit Hyperactivity Disorder (ADHD), which is a common condition in children characterized by symptoms such as inattention, hyperactivity, and impulsivity.
- 2. **Behavioral Observations:** The preschool teachers observe that the patient is more active, frequently interrupts, and is very forgetful. These behaviors align with the symptoms of ADHD, which often include restlessness, difficulty maintaining attention, and impulsive actions.
- 3. **Developmental History:** The boy has met all developmental milestones, suggesting that his symptoms are not due to a developmental delay or intellectual disability. This further supports the likelihood of ADHD, as it typically does not present with delays in development.
- 4. **Environmental Factors:** The dysfunctional family environment (mother with major depressive disorder and father with Graves' disease) could contribute to behavioral issues. However, ADHD is primarily a neurodevelopmental disorder, and the family environment factors may exacerbate symptoms but do not cause them.
- 5. **Exclusion of Other Conditions:** The acute otitis media episodes are unlikely to be related to the current symptoms, as they were resolved. There is no evidence of neurological conditions like Rett syndrome or other severe disorders that could explain the symptoms.
- 6. **Treatment Consideration:** The most appropriate next step in managing ADHD is behavioral therapy, which helps address the behavioral symptoms and teaches strategies for better attention, impulse control, and social interactions.

Conclusion:

The most appropriate next step in treatment is behavioral therapy, as the symptoms are consistent with Attention Deficit Hyperactivity Disorder (ADHD).

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<answer>
Behavior therapy
</answer>